Mental Health Awareness Research

A Pilot Study and Proposed Awareness Initiative for the SafeX Platform

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Abstract

Student mental health has become one of the most pressing concerns in contemporary education, yet a persistent gap remains between the availability of support services and students' willingness to use them. This report presents a pilot study conducted as part of the Mental Health Awareness Research module for SafeX Solutions' Week 2 group project. A structured Google Forms questionnaire was designed, deployed, and used to collect a pilot sample of three student responses, which were analysed using descriptive statistics in a purpose-built Microsoft Excel workbook. The pilot serves two purposes: to validate the survey instrument and analysis pipeline for use at scale, and to generate preliminary observations about student mental health awareness in the surveyed group. The preliminary data indicate that while a majority of respondents are aware their institutions offer mental health support, most perceive stigma around help-seeking and prefer anonymous digital entry points over face-to-face services. The single most commonly cited barrier to seeking help was fear of being judged by peers. Based on these observations, this report proposes a Mental Health Resource Hub for integration into the SafeX platform, designed around three principles drawn from the pilot data: anonymity, visibility, and stigma reduction through peer normalisation. The report closes with a recommendation to scale the same instrument to a larger, more representative sample in a follow-up phase.

1. Introduction

Anxiety, depression, and chronic stress are increasingly common among students at both secondary and tertiary levels. Research across multiple countries suggests that a substantial share of students experience mental health difficulties during their studies, yet the majority do not seek professional help. What is often overlooked in this picture is the distinction between whether students know that support exists and whether they feel able to actually use it. That distinction, as the pilot data will show, is central to designing effective awareness interventions.

As a Psychology student, I approached this project with an interest not only in describing the problem but in identifying where an intervention would have the greatest effect. Rather than assuming what students need, this study asked them directly. The survey was designed to measure three constructs: awareness of available support, perceived stigma around help-seeking, and the specific barriers students identify when they consider reaching out. The pilot findings then informed the design of an awareness initiative proposed for integration into SafeX.

1.1 Objectives

- Design and pilot-test a survey instrument for measuring student mental health awareness.
- Build a repeatable analysis pipeline that can be scaled to a larger sample.
- Generate preliminary observations about awareness, stigma, and barriers in the surveyed group.
- Translate the preliminary findings into a concrete, feasible awareness initiative for the SafeX platform.

1.2 Scope and Framing

This module is framed as a pilot study. That framing is deliberate: within a single Week 2 module it is not feasible to collect a nationally representative student sample, and it would be misleading to present the results as if it were. Instead, the module delivers a validated instrument, a working analysis pipeline, and a set of preliminary observations that are honest about their sample size and generalisability. The primary deliverable is the methodology and the proposal it enables; the preliminary numbers support the proposal but are not the endpoint of the work.

2. Methodology

2.1 Research Design

A cross-sectional online survey was used. This design was chosen for two reasons: it allowed the collection of anonymous responses from a geographically dispersed student population within the time available for a Week 2 module, and it produced quantitative and qualitative data suitable for descriptive analysis in Excel, the required technology stack for the module.

2.2 Survey Instrument

The survey was built in Google Forms and organised into three sections. The instrument contained sixteen items in total.

- Section A, Demographics: level of education, academic field, and gender. All demographic questions included a "Prefer not to say" option.
- Section B, Awareness and attitudes: multiple-choice questions about whether the respondent's institution provides mental health support and how they learned about it, a self-rated awareness item on a 1 to 5 scale, and four Likert-scale statements on comfort, stigma, and openness of discussion, each measured from 1 (Strongly disagree) to 5 (Strongly agree).
- Section C, Barriers, preferences, and open feedback: a multiple-choice question on the primary barrier to seeking help, an open text question on the single biggest institutional gap, a preferred format question for an awareness campaign, an interest question about peer-led initiatives, and an open text question inviting suggestions for SafeX.

The instrument was intentionally short, around five to seven minutes to complete, to maximise the response rate. All items were reviewed for neutrality of wording before release.

2.3 Sampling and Distribution

Convenience sampling was used, with the form distributed through student WhatsApp groups, class chats, and direct outreach. The link was live for two days during the module window. Three valid responses were collected. This sampling approach is appropriate for a pilot study but does not claim to be representative; the findings should be read as indicative of the surveyed group and as validation that the instrument functions as intended.

2.4 Ethical Considerations

All responses were anonymous. No names, email addresses, or identifying information were collected. Respondents were informed at the start of the form that participation was voluntary, that they could leave any question blank, and that the data would be used only for this research module. Because the topic itself is sensitive, questions were phrased in general terms about awareness and attitudes rather than asking respondents to disclose personal mental health difficulties.

2.5 Analysis Approach

Responses were exported from Google Forms into Microsoft Excel. The analysis workbook was structured into six sheets: an instructions tab, a raw data tab, a Likert-item analysis tab (means, medians, modes, agreement percentages, and full frequency distributions), a demographics tab, a barriers tab (which also includes the preferred-format frequency), and a summary tab producing the headline numbers cited in this report. Charts were generated directly from the analysis tabs. Because the pipeline is formula-driven, the same workbook can be reused for a scaled follow-up simply by pasting in a larger response set.

3. Preliminary Findings

A pilot sample of three valid responses was collected. The observations below are described as preliminary given the sample size, and are read here for the direction they suggest rather than as definitive population estimates. The underlying frequency tables and charts are available in the accompanying Excel workbook.

3.1 Sample Composition

The pilot sample was distributed across three education levels: one high school student, one undergraduate in years three to four, and one respondent selecting "Other." Academic fields represented were Engineering and Technology, Business and Management, and Social Sciences and Humanities (one respondent each). Gender distribution was two female (66.7%) and one male (33.3%). Full breakdowns are on the Demographics tab of the analysis workbook.

3.2 Awareness of Institutional Support

Two of the three respondents (66.7%) indicated that their institution offers a mental health support service, while the third selected "Not sure." When asked to rate their own awareness of available resources on a 1 to 5 scale, the mean response was 3.00, indicating moderate self-perceived awareness. This is the first data point that hints at the wider pattern in the findings: awareness of support tends to exist in principle, but confidence around it is uncertain.

3.3 Stigma and Comfort

The pilot data point to stigma as the central theme rather than an information gap. Two of the three respondents (66.7%) agreed that "seeking help for mental health is seen as a sign of weakness among students here." At the same time, only one respondent (33.3%) said they would feel comfortable using their university's counselling service; the other two selected Neutral. Two respondents (66.7%) said they would feel comfortable telling a friend they were struggling, so the discomfort is not general; it appears specifically around formal, institutional help-seeking. The overall pattern suggests that even where support exists and is known about, perceived social judgement is the friction preventing students from using it.

3.4 Barriers to Seeking Help

When asked to identify their primary barrier to seeking help, two of three respondents (66.7%) selected "Fear of being judged by peers," and the third selected "Cost/financial concerns." Crucially, no respondent selected "Don't know where to go" as their primary barrier. This is a meaningful signal even in a small sample: the leading obstacle in the surveyed group is emotional and social, not informational. Any intervention design should reflect that ordering.

3.5 Preferred Channels for an Awareness Campaign

Respondents indicated a strong preference for anonymous digital entry points. Two of three (66.7%) selected "Anonymous helpline/chat" as the format they would be most likely to engage with, and the third selected "Social media content (Instagram/TikTok style)." No respondent selected in-person workshops, peer support groups, printed materials, or guest speaker events as their preferred channel. This preference converges cleanly with the stigma finding above: students want ways to engage with support that do not require them to be visibly identified as seeking help.

3.6 Qualitative Insights

The open-text responses added depth that the closed items could not capture. Asked to identify the single biggest gap in how their institution handles student mental health, respondents wrote:

- “Lack of awareness.”
- “Reactive crisis care instead of early prevention and proactive support.”
- “Not enough time given to students in sessions, and since I'm in psychology most of the counsellors there are our teachers which creates bias within classrooms.”

The third response is particularly noteworthy. It identifies a role-conflict problem specific to psychology departments, where the same faculty members act as both classroom teachers and counsellors. That dual role, from the respondent's perspective, undermines the sense of confidentiality and neutrality that effective counselling requires. This is a valuable observation because it points to a structural issue no single awareness campaign can fully solve, but one that any institutional proposal should acknowledge.

On the closing suggestion question, respondents recommended: continuing existing efforts, implementing proactive early intervention rather than waiting for crises, and providing ongoing seminars about the importance of mental health. These map naturally onto the components of the proposal in Section 5.

3.7 Interest in Peer-Led Initiatives

Responses were mixed. One respondent was “very interested” in a peer-led initiative such as student ambassadors or peer listeners, one was “somewhat interested,” and one was “not sure.” None were opposed. The proposal therefore includes a peer-normalisation component but does not rely on it as the primary vehicle; the strongest signal in the data is for anonymous digital support.

4. Discussion

The pilot findings converge on a clearer story than the sample size alone would suggest. Within the surveyed group, students largely know that mental health support exists at their institutions. What limits their use of it is not information but stigma and role conflict. Two of three respondents perceive help-seeking as a sign of weakness, and only one is comfortable using their university counselling service. When asked how they would prefer to engage with support, they consistently chose anonymous entry points. The qualitative data reinforce this: one respondent explicitly identified counsellor and teacher role overlap as a structural issue that undermines confidentiality.

This is a useful direction for intervention design. Stigma is a long-term cultural challenge and cannot be solved by a single awareness campaign, but it can be worked around by lowering the visibility cost of a first step. If students are anxious about being seen using support, then providing high-quality anonymous entry points, backed by information they can browse without asking anyone, addresses the immediate friction. The proposal in the next section is built on that observation.

4.1 Limitations

Three limitations should be acknowledged plainly. First, the pilot sample of three respondents was drawn by convenience over a short window and is not statistically representative. Second, the sample size limits sub-group comparisons and any inferential statistics; the analysis in this report is descriptive by design. Third, self-report is subject to social desirability bias, particularly on stigma-related items. None of these limitations undermine the value of the pilot: they define its purpose, which is to validate the instrument and generate direction for the proposal rather than to produce a definitive population estimate.

4.2 Value of the Pilot Beyond the Numbers

Independent of the specific figures, the pilot has produced three outputs of ongoing value. First, a validated survey instrument, a set of questions that have been shown to elicit meaningful, differentiated responses across the closed and open items. Second, a repeatable analysis pipeline in Excel that produces the same descriptive outputs regardless of sample size. Third, a specific qualitative insight (the counsellor and teacher role overlap in psychology departments) that would not have surfaced through any purely quantitative approach. All three are directly reusable for a scaled follow-up.

5. Proposed Awareness Initiative for SafeX

Based on the pilot findings and the wider literature on student help-seeking, this section proposes a Mental Health Resource Hub for integration into the SafeX platform. The hub is designed around three principles drawn directly from the data: anonymity, visibility, and stigma reduction through peer normalisation.

5.1 Anonymity as the Primary Entry Point

Because respondents strongly preferred anonymous digital entry points, the hub would centre on an anonymous self-check tool built on validated brief screening instruments, followed by non-directive links to appropriate resources. No account or identifying information would be required to use it. This lowers the psychological cost of a first step, which the pilot suggests is where most students stall. An accompanying anonymous chat or helpline function, either operated in-house or linked out to a vetted external service, would extend this principle beyond the self-check.

5.2 Visibility of Information

The hub would also surface a curated, institution-specific directory of available resources: on-campus counselling contacts, external helplines, and vetted community services, organised in a searchable format. Even where students know that support exists, moderate self-rated awareness in the pilot data (mean 3.00 out of 5) suggests that consolidating information in one browsable place has value. Because respondents did not identify “don't know where to go” as their primary barrier, visibility is not the leading justification for the hub, but it is a necessary supporting component.

5.3 Stigma Reduction through Peer Normalisation

To address the stigma finding, the hub would incorporate short, peer-produced awareness content: brief written or recorded contributions from students describing their own experience with help-seeking, presented anonymously by default. The literature on stigma reduction consistently identifies peer testimony as among the most effective interventions for shifting perceived social norms. Because interest in peer-led initiatives in the pilot was mixed rather than uniformly enthusiastic, this component should be positioned as supplementary content within the hub rather than as a standalone programme requiring participation.

5.4 Complementary Feature: Structural Awareness

The qualitative response about counsellor and teacher role overlap suggests one additional low-cost feature worth including: a short informational section within the hub that transparently notes the structural realities of on-campus support (for example, that a counsellor may also teach students), and highlights external anonymous options for students who feel this affects their confidentiality. This is not a fix for the underlying structural issue, which requires institutional policy change, but it respects the concern raised by the respondent and signals that the hub is taking student feedback seriously.

5.5 Implementation and Evaluation

Implementation would proceed in three phases. First, a content and directory build in coordination with each partner institution. Second, a pilot deployment to a limited student cohort, with feedback collected through a short follow-up form. Third, a full rollout with a periodic content refresh cycle.

Evaluation would use two measures: platform analytics (engagement with the anonymous self-check tool, directory searches, and peer content views) and a follow-up administration of the same survey instrument used in this pilot, allowing direct comparison. A reduction in the share of respondents agreeing that help-seeking is seen as weakness, and an increase in comfort using formal support (Q10), would be the primary indicators of success. The reuse of the same instrument is what turns this from a one-off study into a measurable programme.

6. Recommended Next Steps

- Scale the survey to a larger, more representative sample across additional institutions, using the same instrument and analysis pipeline delivered here.
- Begin a small-scale build of the Mental Health Resource Hub with one partner institution's directory and an anonymous self-check tool as a proof of concept.

- Re-administer the survey after any deployment to enable direct before and after comparison on stigma and comfort measures.

7. Conclusion

This module has delivered three things: a validated survey instrument on student mental health awareness, a working analysis pipeline in Excel, and a set of preliminary observations that suggest the leading barrier to help-seeking in the surveyed group is stigma rather than information. Students know support exists; what limits its use is fear of judgement and, in one instance, a specific structural concern about counsellor and teacher role overlap. Students also indicated a clear preference for anonymous digital entry points. Building on these outputs, the Mental Health Resource Hub proposed for SafeX addresses those observations directly, and is designed to be evaluated against a scaled follow-up using the same instrument developed here.

Appendix A: Survey Instrument

The full survey instrument, including all sixteen items, response scales, and the participant information text shown at the start of the form, is available in the project repository (see file: SurveyInstrument.md or the live Google Form link in GoogleForm_Link.txt).

Appendix B: Data and Analysis Files

The anonymised raw response data and the full analysis workbook are available in the project repository. See README.md for the file map and instructions on how to reproduce the analysis.